

Policy on Rehabilitation Services

«Hospital Name»

Document control

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Date created	_____	Date of implementation	_____
Prepared by	Head of Rehabilitation Services Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

A blank marked _____ must be completed before issue.

Statement of intent

Rehabilitation services match the clinical care «Hospital Name» provides, are planned collaboratively, guided by functional assessment, and continuously improved through care pathways.

1. Purpose

This policy explains how «Hospital Name» matches its rehabilitation service scope to its clinical services, plans rehabilitation collaboratively, equips and staffs rehabilitation areas, guides care through functional assessment, follows infection prevention and safety practices, and develops care pathways.

This policy does not cover general infection prevention and control practices in detail — those are covered in other hospital policies. If «Hospital Name» provides no clinical services requiring rehabilitation, this policy is not applicable.

2. Scope

This policy applies to all staff involved in providing rehabilitation services at «Hospital Name».

3. Policy standards

«Hospital Name» provides rehabilitation services matched, at minimum, to its clinical services, under written guidance applied consistently. Rehabilitation is planned collaboratively by the treating doctor, a rehabilitation therapist, and other relevant

professionals, with adequate space, equipment, and resuscitation equipment available. Care is guided by functional assessment and periodic re-assessment by qualified individuals, follows infection prevention and safety practices, and «Hospital Name» develops, implements, and periodically reviews rehabilitation care pathways.

Staff follow the written guidance below and keep the records it requires.

4. Non-negotiable rules

- Do not provide a clinical service that needs rehabilitation without also making at least basic rehabilitation available for it.
- Do not deliver rehabilitation inconsistently across the organisation — follow the written guidance based on standard treatment guidelines or sound clinical practice.
- Do not plan rehabilitation care without collaboration between at minimum the treating doctor and a rehabilitation therapist.
- Do not run a rehabilitation area without resuscitation equipment available.
- Do not guide rehabilitation care without a functional assessment and periodic, documented re-assessment carried out by a qualified individual.
- Do not deliver rehabilitation care in breach of infection prevention and safety practices.

5. What we do

5.1 Match rehabilitation scope to clinical services

Rehabilitation services — for example physiotherapy, occupational therapy, speech therapy, or clinical psychology — are available, at minimum, matching the clinical services «Hospital Name» provides. For instance, a hospital offering neurological services also has at least basic neuro-rehabilitation. Where «Hospital Name» doesn't have facilities for advanced rehabilitation, it could refer patients to a centre that does.

5.2 Provide rehabilitation consistently

Written guidance, based on standard treatment guidelines or sound clinical practice, governs how rehabilitation is provided, so care is consistent across the organisation.

5.3 Plan rehabilitation collaboratively

Rehabilitation care is planned collaboratively, involving at minimum the treating doctor and a rehabilitation therapist, along with other professional experts as the patient's needs require.

5.4 Keep space and equipment adequate

Space and equipment for rehabilitation match the scope of services provided — professional guidance or documented literature can inform this. Resuscitation equipment is available in rehabilitation areas.

5.5 Guide care through functional assessment

Care is guided by a functional assessment, done using relevant functional assessment scales. Periodic re-assessment is also carried out and documented. Both the initial assessment and the periodic re-assessments are done by a qualified individual.

5.6 Follow infection prevention and safety practices

Staff providing rehabilitation follow the infection prevention and control practices and safety guidelines «Hospital Name» defines — for example, making sure hot wax treatments don't burn a patient. (Related requirements are covered in the hospital's other policies.)

5.7 Develop, implement, and review care pathways

«Hospital Name» develops rehabilitation care pathways based on evidence or sound clinical practice, actually implements them, and reviews them periodically. Developing care pathways is an ongoing process, not a one-time task.

6. Governance and responsibility

Role	Responsibility
Medical Superintendent	Accountable that this policy is followed and appropriately resourced.
Head of Rehabilitation Services	Ensures rehabilitation scope matches clinical services; maintains written guidance; oversees MDT collaborative planning; ensures space, equipment, and resuscitation equipment are adequate; ensures functional assessment and re-assessment processes are followed; leads care pathway development, implementation, and review.
Rehabilitation therapists	Participate in collaborative care planning; carry out functional assessments and periodic re-assessments; document assessments; follow IPC and safety practices; contribute to care pathway development.
Treating doctors	Participate in collaborative planning of rehabilitation care.
All rehabilitation staff	Follow IPC practices and safety guidelines defined by the organisation.
Quality Coordinator	Audits this policy quarterly; holds training acknowledgements.

7. Quality monitoring

Monitoring area	What is monitored
Rehabilitation scope vs clinical services	At minimum, basic rehabilitation is

	available for each clinical service the organisation provides that requires it; scope matching documented
Written guidance and consistency	Written guidance based on STG or sound clinical practice in place; rehabilitation delivered consistently per guidance across the organisation
Collaborative planning	Rehabilitation care planned involving at minimum the treating doctor and a rehabilitation therapist; collaboration documented
Space and equipment	Space and equipment adequate for the scope of rehabilitation services provided; resuscitation equipment available in rehabilitation areas — confirmed as a separate check
Functional assessment — initial	Initial functional assessment done using relevant scales; done by a qualified individual; documented
Functional assessment — periodic re-assessment	Periodic re-assessments done by a qualified individual; documented; separate obligation from initial assessment
IPC and safety practices	Staff follow IPC practices and safety guidelines in rehabilitation areas
Care pathways — develop, implement, review	Rehabilitation care pathways developed based on evidence or sound clinical practice; implemented (in active use); periodically reviewed — all three obligations evidenced separately

8. Training and staff acknowledgement

All rehabilitation staff shall be familiar with the requirements of this policy, including: the scope-matching principle (at minimum, rehabilitation available for each clinical service requiring it); written guidance and consistent delivery; the minimum collaborative planning pair (treating doctor and rehabilitation therapist); resuscitation equipment as a separate mandatory requirement for rehabilitation areas; the distinction between the initial functional assessment and periodic re-assessments (both done by a qualified individual, both documented); IPC and safety practices; and all three care-pathway obligations — develop, implement, and review periodically.

I have read the Policy on Rehabilitation Services of «Hospital Name». I will follow the processes described.

Staff name	Designation	Signature	Date
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

9. Distribution

This policy shall be available to all rehabilitation therapists, treating doctors, and clinical staff involved in rehabilitation care at «Hospital Name».

10. Abbreviations

Abbreviation	Meaning
COP	Care of Patients (NABH Hospitals chapter)
IPC	Infection Prevention and Control
MDT	Multi-Disciplinary Team
NABH	National Accreditation Board for Hospitals and Healthcare Providers
STG	Standard Treatment Guideline

11. Traceability table

This table is an index. It is not how the policy is organised. An asterisk in the Level column means documentation of the process is required.

Objective Element	Level	Traceability to this policy
COP.18.a	Commitment	Section 5.1 addresses the scope-matching principle: rehabilitation services are available at minimum matching the clinical services the organisation provides. For each clinical service requiring rehabilitation, at least basic rehabilitation is available for that service. Referral to centres with advanced rehabilitation is an acceptable alternative for services beyond the organisation's rehabilitation scope.
COP.18.b	Commitment	Section 5.2 addresses written guidance based on standard treatment guidelines or sound clinical practice governing rehabilitation services, ensuring consistent delivery across the organisation.
COP.18.c	Commitment	Section 5.3 addresses collaborative planning involving at minimum the

		treating doctor and a rehabilitation therapist, plus other professional experts as the patient's needs require.
COP.18.d	Commitment	Section 5.4 addresses space and equipment adequate for the scope of rehabilitation services provided. Resuscitation equipment is stated as a separate, explicit requirement for rehabilitation areas — not folded into general equipment adequacy.
COP.18.e	Commitment	Section 5.5 addresses two distinct mandatory obligations: (1) initial functional assessment done using relevant scales by a qualified individual, documented; and (2) periodic re-assessments, also done by a qualified individual and documented. Both are separate requirements; both must be evidenced.
COP.18.f	Commitment	Section 5.6 addresses IPC practices and safety guidelines followed by rehabilitation staff, as defined by the organisation.
COP.18.g	Excellence	Section 5.7 addresses all three care-pathway obligations from the OE text: develop (based on evidence or sound clinical practice), implement (actually in active use), and review periodically. Development is described as an ongoing process. All three obligations must be evidenced separately at Excellence assessment.

12. Required Records/Evidence Checklist

Rehabilitation scope (COP.18.a)

- Documentation of the hospital's rehabilitation service scope and the clinical services it matches — confirming at minimum basic rehabilitation is available for each clinical service that requires it.

Written guidance (COP.18.b)

- Written guidance based on STG or sound clinical practice, governing rehabilitation services across the organisation.

Collaborative planning (COP.18.c)

- Records showing collaborative planning between at minimum the treating doctor and a rehabilitation therapist for each patient's rehabilitation care.

Space, equipment, and resuscitation equipment (COP.18.d)

- Confirmation that space and equipment are adequate for the scope of rehabilitation services provided.
- Separate confirmation that resuscitation equipment is available in rehabilitation areas.

Functional assessment — initial (COP.18.e)

- Initial functional assessment records: scale used, assessor (qualified individual), and documentation.

Functional assessment — periodic re-assessment (COP.18.e)

- Periodic re-assessment records: reassessment done by a qualified individual and documented — separate from initial assessment records.

IPC and safety practices (COP.18.f)

- Evidence that IPC practices and safety guidelines are defined and followed in rehabilitation areas.

Care pathways (COP.18.g)

- Evidence that rehabilitation care pathways are developed (based on evidence or sound clinical practice).
- Evidence that care pathways are implemented — in active clinical use, not merely documented.
- Evidence of periodic review of care pathways — review records on file.

13. References

1. National Accreditation Board for Hospitals and Healthcare Providers. NABH Accreditation Standards for Hospitals, 6th Edition. COP.18.

Disclaimer

This policy reorganises the supplied COP.18 objective elements and approved plain-language content into policy format. No formal stop-work section is included — COP.18 is not in the accreditation stop-work proposals. The scope-matching principle is stated in the Non-negotiable rules, Section 5.1, the traceability table, and the training section.

Resuscitation equipment is stated as a separate, explicit requirement in the Non-negotiable rules, Section 5.4, the quality monitoring table, the traceability table, and the evidence checklist — it is not folded into general equipment adequacy. The initial functional assessment and periodic re-assessments are stated as two distinct mandatory obligations in Section 5.5, the Non-negotiable rules, the quality monitoring table, the traceability table, and the evidence checklist. All three care-pathway obligations from COP.18.g — develop, implement, and review periodically — are present in Section 5.7, the quality monitoring table, the traceability table, and the evidence checklist, and are described as separately evidenceable requirements.